

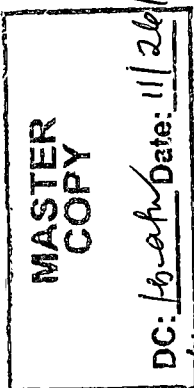
PHILHEALTH CIRCULARNo. 2025-0024**TO : ALL ACCREDITED HEALTH CARE PROVIDERS,
PHILHEALTH MEMBERS, PHILHEALTH REGIONAL
OFFICES AND ALL OTHERS CONCERNED****SUBJECT : Quality Standards on the Screening for Breast Cancer as
Reference of the Corporation****I. RATIONALE**

Republic Act No. 11223 (Universal Health Care Act) mandates PhilHealth to ensure the delivery of quality health services in accordance with evidence-based clinical practice guidelines endorsed by the Department of Health (DOH) and relevant specialty societies.

Quality-assured cancer screening is vital for early detection and timely intervention, leading to reduced disease burden and improved health outcomes. The establishment of uniform standards ensures consistent service delivery, strict adherence to recognized clinical guidelines, and protection against substandard or inappropriate practices, fulfilling PhilHealth's mandate to safeguard the health of all Filipinos.

Breast cancer remains the top cancer that affects Filipinos which account for at least 33,079 (60%) of new cancer cases in the Philippines in 2022. Breast cancer is considered a significant public health issue that countries, especially low-middle income countries, must immediately intervene to reduce the burden of the disease, mitigate social and financial disruptions in families and communities, and improve the overall survival of affected groups. To address the burden of breast cancer, the Philippines established public health cancer screening programs in 2019. However, the screening recall system and logistical limitations have left the breast cancer screening indicators underperforming with only 830,691 (2.5%) women screened for breast mass in 2023.

Through this issuance, PhilHealth reaffirms its commitment to delivering high-quality, accessible, and patient-centered care, and thereby upholding its mandate to protect the health and well-being of every Filipino.



II. OBJECTIVES

This PhilHealth Circular aims to establish the minimum standards of care for breast cancer screening in alignment with the Corporation's Quality Assurance Program.

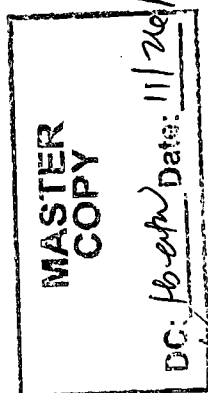
III. SCOPE

This PhilHealth Circular defines evidence-based parameters for eligibility assessment, risk stratification, and the delivery of breast cancer screening services aligned with nationally adopted guidelines and harmonized with international best practices.

This policy covers all PhilHealth-accredited health facilities possessing the capability and the appropriate Department of Health (DOH) license to provide the relevant imaging services. This shall apply to all Filipinos registered to their respective PhilHealth's Yaman ng Kalusugan Program (YAKAP) clinics for the covered outpatient benefits.

IV. DEFINITION OF TERMS

- A. Cancer Screening** – presumptive identification of unrecognized disease in an apparently healthy, asymptomatic, high-risk population by means of tests, examinations or other procedures that can be applied rapidly and easily to the target population.¹
- B. Detectable Pre-Clinical Stage** – the period during which an undiagnosed cancer can be detected through screening before any clinical symptoms appear.²
- C. Diagnostic Procedures** – medical tests performed to determine the underlying disease or condition based on a patient's signs and symptoms. These procedures also guide treatment planning, assess treatment effectiveness, and support prognosis. They may include laboratory examinations (e.g., blood and urine tests), imaging studies (e.g., mammography, computed tomography), endoscopic evaluations (e.g., colonoscopy, bronchoscopy), and tissue sampling through biopsy.³



¹World Health Organization. (2017). Promoting cancer early diagnosis. <https://www.who.int/activities/promoting-cancer-early-diagnosis>

²Smith, R. A., Mettlin, C. J., & Eyre, H. (2003). Key criteria in the decision to screen. In D. W. Kufe, R. E. Pollock, & R. R. Weichselbaum (Eds.), *Holland–Frei Cancer Medicine* (6th ed.). BC Decker. <https://www.ncbi.nlm.nih.gov/books/NBK13727/>

³National Cancer Institute. (n.d.). Diagnostic procedure. NCI Dictionary of Cancer Terms. <https://www.cancer.gov/publications/dictionaries/cancer-terms/def/diagnostic-procedure>

D. Eligibility Assessment – an evaluation process used to determine whether an individual or entity meets the necessary criteria to be eligible for a particular benefit, service, or program.⁴

E. Risk Factor – anything that raises an individual's chances of getting a disease such as cancer.⁵

F. Risk Stratification – an approach clinicians use to make sure their patients get what they need in terms of care and to allocate limited resources to the patients who need them most.⁶

V. POLICY STATEMENTS

A. Eligibility Criteria

1. Breast cancer screening shall be provided to eligible, asymptomatic, high-risk individuals within defined target populations, for diseases with a detectable preclinical stage, where early detection and timely treatment have been shown to improve health outcomes.
2. The target population for breast cancer screening is any of the following:
 - a. Asymptomatic average-risk women aged 50-69 years old;
 - b. Asymptomatic women at high risk (with first-degree relative diagnosed with breast cancer at age 49 or below), aged 30-49 years old;
 - c. Women aged 40-49 years with identified risk factors; OR
 - d. Individuals with breast symptoms (e.g., breast mass, breast or nipple pain, nipple discharge, changes in the size, shape, or appearance of breasts, lumps or swelling in the armpit/axilla or near collarbone).
3. Identified risk factors refer to any of the following:
 - a. Family history of breast cancer in a first-degree relative; OR
 - b. Previous history of breast cancer who did not undergo double radical mastectomy.

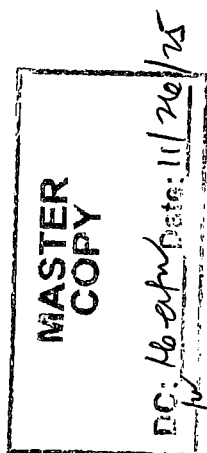
B. Conduct of Screening Process

1. PhilHealth-accredited primary care physicians shall offer breast cancer screening as a preventive health service, guided by risk stratification. Screening shall utilize appropriate diagnostic procedures to detect cancer or precancerous conditions in asymptomatic individuals.

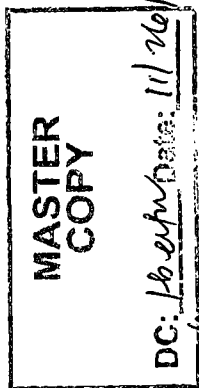
⁴US Insurance Agents. (2025, January 15). What is eligibility assessment? <https://www.usinsuranceagents.com/what-is-eligibility-assessment/>

⁵PhilHealth Circular 2025-0014 PhilHealth Benefit Packages for Selected Outpatient Cancer Screening Tests

⁶Genomind. (2022, October 18). Risk stratification definition and how it affects patient care. <https://genomind.com/patients/population-health-risk-stratification/>



2. All eligible patients, as determined through a standardized eligibility assessment, shall undergo pre-screening counseling by qualified and PhilHealth-accredited healthcare providers to explain the purpose, benefits, risks, and limitations of the cancer screening tests. It should include informed consent as a prerequisite for service provision.
3. Screening shall be performed only for individuals meeting the clinical and eligibility criteria outlined in Section V.A. of this PhilHealth Circular.
4. The eligibility assessment by the accredited primary care physicians shall be documented, detailing relevant patient history, risk profile, and rationale.
 - a. The Client Assessment, Screening, Diagnosis, and Treatment for Cervical-Breast Cancer (CASDT) form released by the Department of Health shall be considered sufficient for the purposes of the eligibility assessment, provided that the data in the CASDT form is also encoded in the electronic medical record of the patient.
5. Screening tests for breast cancer include the following:
 - a. Clinical breast examination (CBE):
 - a.1. Perform CBE for all women starting at age 25, with patient consent.
 - a.2. For pre-menopausal women, the examination should be conducted 5-7 days after the menstrual period. For post-menopausal women, timing of the examination is flexible.
 - b. Mammogram:
 - b.1. A low-dose X-ray imaging procedure of the breast used to detect early changes or abnormalities not palpable on physical examination.
 - b.2. Mammogram shall be conducted every two years for women aged 40 – 69 years old.
 - c. Breast ultrasound:
 - c.1. A non-invasive imaging procedure that uses sound waves to visualize breast tissue. Ultrasound is particularly useful for select patients who have dense breasts and difficult to perform CBE.
 - c.2. Breast ultrasound may be conducted every year for high-risk women in conjunction with mammogram aged 40 – 69 years old.



6. Breast cancer screening tests shall be performed exclusively by trained and credentialed personnel in health facilities licensed by the Department of Health and accredited by PhilHealth.
7. Test results shall be explained immediately to the patient, with proper interpretation including post-screening counseling.
8. Any abnormal screening result shall prompt timely referral to appropriate specialists for confirmatory diagnostic procedures or treatment services, in accordance with the primary care physician's established referral pathways. All referrals are to be documented.

C. Monitoring and Evaluation

1. The health care provider shall be bound by the provisions of the Performance Commitment and subject to the rules on monitoring and evaluation of performance as provided in PhilHealth Circular No. 2018-0019 Health Care Provider Performance Assessment System (HCPPAS) Revision 2.
2. Standards of care issued by authorized agencies/organizations shall be regularly monitored. As deemed necessary, a revision of this policy shall be made. Any updates resulting from the review shall be disseminated in another PhilHealth Circular.

VI. PENALTY CLAUSE

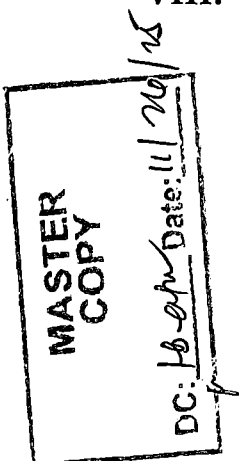
Any violation of this PhilHealth Circular shall be dealt with and penalized in accordance with pertinent provisions of R.A. No. 11223 and R.A. No. 10606 (National Health Insurance Act of 2013), and their respective Implementing Rules and Regulations.

VII. SEPARABILITY CLAUSE

In the event that any part or provision of this PhilHealth Circular is declared unauthorized or invalid by any Court of Law or competent authority, those provisions not affected by such declaration shall remain valid and effective.

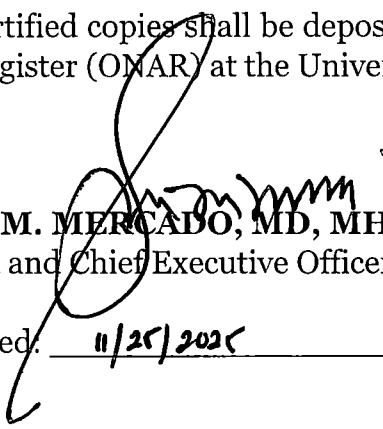
VIII. REPEALING CLAUSE

This PhilHealth Circular repeals, amends, or modifies Annex A in PhilHealth Circular No. 2025-0014 PhilHealth Benefit Packages for Selected Outpatient Cancer Screening Tests that are contrary or inconsistent to its provisions.



IX. DATE OF EFFECTIVITY

This PhilHealth Circular shall take effect fifteen (15) days after its publication in the Official Gazette or in any newspaper of general circulation. Three (3) certified copies shall be deposited to the Office of the National Administrative Register (ONAR) at the University of the Philippines Law Center.


EDWIN M. MERCADO, MD, MHA, MMSc
President and Chief Executive Officer

Date signed: 11/25/2025

